



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان صلاح محمد احمد
المحافظة : الجيزة
الحى / المدينة : اوسيم
العنوان : -
رقم البطاقة : 28508202102548 رقم التليفون : 01227908765
العمر (تاريخ الميلاد) : 20-08-1985 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و
بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان صلاح محمد احمد National ID: 28508202102548	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

ECHO REPORT

NAME: Wagdy DATE: 18 / 1 / 2026
ID NUMBER: 877 / 2025 SEX: ♀ AGE: 40

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve Normal
Aortic Valve Normal
Tricuspid Valve Normal
Pulmonary Valve Normal
Right Side and Systolic Function TAPSE 2.3
Intracardiac Masses and Effusion

DR. [Signature]

Name:	حسان صلاح محمد	
MR. :	2980	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was diagnosed with left-sided breast cancer, clinical stage T3 N1, grade IDC, estrogen receptor 8/8, progesterone receptor 8/8, HER2 23+. Initial treatment included a left mastectomy on 19 October 2025, followed by adjuvant chemotherapy (CRYPTO YPN0/8) and tamoxifen. Zoladex was started in December 2025. Pathology reported an estrogen-fraction (EF) of 62 % on 18 January 2026, and the patient was scheduled to receive Cycle 8 of Phesgo with tamoxifen, with Cycle 9 and Zoladex due on 13 March and Cycle 10 due on 3 April. The patient attended a pharmacy visit for Cycle 9 of Phesgo combined with tamoxifen and Zoladex. The patient has completed Cycle 9 of Phesgo, Tamoxifen, and Zoladex. During multidisciplinary team discussion, the patient was planned to receive Cycle 11 of Phesgo and tamoxifen, with a shift to an aromatase inhibitor while continuing Zoladex because of high-risk disease (EF 65 %). The patient has completed Cycle 11 of Phesgo and Tamoxifen. At the subsequent oncology visit, the patient was noted to be on Cycle 10 of Phesgo and tamoxifen with supportive therapy. Chest CT performed by pharmacy review showed no metastatic disease, but a left apical consolidation consistent with inflammatory changes. The patient has completed Cycle 10 of Phesgo and Tamoxifen. On follow-up, the patient's EF was 65 % as of 15 July. Transvaginal ultrasound measured an endometrial thickness of 5 mm. The patient was scheduled for Cycle 12 of Phesgo, tamoxifen, and Zoladex, with the next dose due on 3 September, and was prescribed methyltechno for one month for symptom management. The patient has completed Cycle 12 of Phesgo, Tamoxifen, and Zoladex. The most recent visit documented preparation for Cycle 13 of Phesgo, tamoxifen, vitamin D supplementation, methyltechno, and Zoladex, with administration planned on 3 September. The patient has completed Cycle 13 of Phesgo, Tamoxifen, and Zoladex.		
تحريراً في ,		
	الوقت :	التاريخ : 21-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		



مصرية سكان
MASRIA SCAN RADIOLOGY & LABS

Patient Name: Hanan Salah Mohamed	Age: 39 Years
Patient ID: 147664	Exam Date: 13/04/2025
Ref.Phy.Prof. Dr:	

Tc^{99m} MDP DUAL PHASE BONE SCAN

Procedure:

- Early blood pool and delayed osseous phase whole body scans were obtained 5 minutes and 2 hours respectively post injection of 20 mCi of the tracer.

Previous Imaging:

- No relevant imaging available for comparison.

Findings:

- No abnormal focal active or cold osseous lesions could be detected all over the scanned body to account for serious osseous lesions
- The early blood pool images were unremarkable with no hypo vascular or hyper vascular areas depicted.

Opinion:

- *The skeleton is free from bone metastases*

Best Regards,
Prof. Dr. Maha Abd Elkareem
Professor of Nuclear Medicine
kasr al ainy, cairo University

For Further medical information or if you have any questions please do not hesitate to contact me.

Dr. Ahmed Waheed

01005055422



Patient Name: Hanan Salah Mohamed Ahmed	Date : 18/03/2025
Exam : C.T ABDOMEN & PELVIS	Age : 39 Year(s)
Medical ID: 877/2025	Pat. ID: 98498

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

→ Newly discovered left breast cancer.

Findings:

I Chest:-

- Left breast heterogenous parynchema wit calcific foci.
- Left enlarged axillary lymph nodes at level I, II and III the largest is seen at level I measures about 2.1x2.7 cm (Tx AP diminsion, respectively)
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definte focal lesions (on non contrast base) or contour irregularities.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - **Evidence of tow hyperdens gall bladder stones.**
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas, and both suprarenals.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - IUCD is seen.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
- * Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

- No CT evidence of pulmonary , ossoeous depoists or heptic focal lesions on noncontrast basis.
- No evidence of pulmonary, hepatic or osseous metastatic deposits.
- Gallbladder stones as described above.

Dr. Khawla Ahmed

Prof. Menna Talla Hanafy, M.D.

Women's Health Outreach Program

Name: Hanan Salah Mohammad

Date: 18-02-2025

Bilateral Full field Digital Mammography

Clinical summary: 36 years old lactating lady with negative family history complaining of left breast lumpiness of six months duration.

Technique

Medio-lateral oblique and CC views. High-resolution ultrasound

Detailed Findings

Right breast

- Heterogeneous breast parenchyma that may obscure small lesions (ACR density c)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated echogenic mammary parenchyma with diffusely dilated ducts reaching 7 mm in caliber showing finely turbid fluid content (milk), in keeping with lactation.
- No solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Heterogeneous breast parenchyma that may obscure small lesions (ACR density c)
- UOQ Segmental coarse heterogeneous suspicious microcalcifications seen overlying an area measuring 14 x 10 x 9 cm and reaching the nipple. Within such area multiple irregular speculated high density mass lesions are noted with surrounding distortion
- Ultrasound demonstrated echogenic mammary parenchyma with diffusely dilated ducts reaching 10 mm in caliber showing finely turbid fluid content (milk) in keeping with lactation.
- 11- 4 o'clock, through the whole thickness of the breast, showed altered parenchyma, prominent ducts with suspicious microcalcifications within seen reaching the nipple along with multiple irregular anti-parallel speculated solid mass lesions showing posterior acoustic shadowing, moderate internal vascularity and microcalcifications within, the whole area measured about 13 x 10 x 8 cm
- The largest ones among those masses are seen as follows:

- Index mass: 11-12 o'clock, 7.7 cm from nipple, 9 mm deep to the skin measuring 32 x 27 mm
- Mass B: 1 o'clock, 6 cm from nipple, 7 mm deep to the skin, measuring 26 x 31 mm
- Mass C: 2 o'clock, 8 cm from nipple, 5 mm deep to the skin, measuring 16 x 15 mm
- Mass D: 3 o'clock, 7 cm from nipple, 10 mm deep to the skin, measuring 19 x 12 mm
- Mass F: 4 o'clock, 5 cm from nipple, 10 mm deep to the skin, measuring 15 x 9 mm
- Multiple variable sized satellites are seen in-between those lesions ranging from 4 mm - 10 mm in greatest dimensions.
- Normal skin/subcutaneous tissue and nipple/areolar complex
- Multiple pathological enlarged lymph nodes: about 5; on level 1, the largest measured 20 x 16 mm, while few were noted at level 2, largest measured 6 x 7 mm, no pathological nodes detected at level 3 or supra-clavicular.

Impression:

Right breast (BIRADS 2): Lactation associated changes,

Left breast (BIRADS 5): Lactation associated changes and Multi-centric pathological process as described along with pathological axillary level 1 & 2 lymph nodes. US guided Tru-cut biopsy was taken at same setting from 2 o'clock lesion.

Recommendations:

Left breast: Histopathological correlation.

*Much Obligated,
Dr. Amal Hegazi, M.Sc.*

Patient Name : الأستاذة / د. حنان صلاح محمد أحمد

Patient No. : 28508202102548

Path No. : 734/25

Sex : Female

Age : 39 Y

Referring Doc :

Request No. : 13003803

Request Date : 19/02/2025

Verification Date : 11/03/2025

Financial Category : نفقة الدولة - اوراد دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast multi-centric masses.

Nature of Specimen: Tru-cut biopsy, collected at 18/2/2025, 11:15 am.

Gross Examination:

- Three grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: Focal DCIS
- Perineural invasion: Not detected.
- Angiolymphatic invasion: Not detected.
- Microcalcifications: Not detected.
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 17% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).

Verified by :

Dr.Toka Medhat

Printing Date : 13/03/2025 12:33:26

Page 1 of 2

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأملاكه / حنان صلاح محمد احمد	Request No. : 13003803
Patient No. : 28508202102548	Request Date : 19/02/2025
Pain No. : 734/25	Verification Date : 11/03/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 39 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).
- Ki67: Positive in 17% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Amal Hareedy

Dr. Menna Nabil

Dr. Yasmine Essam

Verified by :

Dr.Toka Medhat



pp

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name: حسان صالحDate: 9/4/ 2025

Age:

ANVCo

EF	<u>58%</u>	54-74%	PWT	<u>6-8</u>	0.6-1.1cm
LVEDD	<u>4.72</u>	3.5-5.7cm	AO	<u>2.92</u>	2.0-3.7cm
LVESD	<u>3.28</u>	2.2-4.0cm	LA	<u>3.2</u>	1.9-4.0cm
FS	<u>30%</u>	25-45%	RV	<u>~</u>	0.7-2.3cm (supine)
SWT	<u>0.93</u>	0.6-1.1cm	AO.CUSP	<u>~</u>	1.5-2.6cm

LV dimensions: NOLV wall thickness: NOLV diastolic function: NOSWMA. NO ab leftLA size NOAortic root dimension NOMitral Valve NO PGAortic valve NO PGTricuspid valve TRIVU TRPulmonary valve TRWU PRRight side size and systolic function NOIntracardiac masses and thrombi NOPericardium NOIVC diameter and collapsibility collapsed

Conclusion:

DI/

Patient Name: Hanan Salah Mohamed Ahmed	Date : 23/09/2025
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 877/2025	Pat. ID: 98498

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 40-year-old female with Left IDC grade II , underwent NACT , follow up study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UOQ segmental coarse heterogenous calcifications over an area measuring 14 x 10 x 9 cm with marked distortion and multiple small scattered high density speculated mass lesions.
- **Ultrasound** demonstrated 11-4 o'clock, still noted altered parenchyma with prominent ducts showing calcifications within, reaching the nipple and multiple irregular spiculated heterogenous hypoechoic solid mass lesions with mild internal vascularity on color Doppler study yet showing regressive course regarding their size, largest measured 12 x 7 mm
- Normal skin/nipple/areola complex.
- Still noted three level 1 pathological axillary LNs showing cortical thickness reaching 5 mm , yet are regressive regarding their size.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): Partial radiological response regarding the previously reported malignant process.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Patient Name :	الأستاذة / حنان :صلاح محمد أحمد	Request No. :	13007410
Patient No. :	28508202102548	Request Date :	19/10/2025
Path No. :	3187/25	Verification Date :	11/11/2025
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	40 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature Of Specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 22x18x5 cm, with covering skin measured 18x12 cm. N&A looked unremarkable grossly.
- * Serialling showed a tumor bed site 7x5x2 cm, located 3 cm from the least deep margin.
- * Axillary fat measured 9x7x2 cm, entangling only 18 lymph nodes, the largest was 1x1x0.5 cm.
- * 22 Blocks:
- 1:7- T.B.S., 8- N&A., 9- Deep margin., 10:13- ALNs., 14:22: Regross from T.B.S.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor (After extensive sampling of the tumor bed site).
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy with severe sclerosis & microcalcification.
- * Nipple & areola: Free.
- * Resection Margin: Free.
- * Axillary lymph nodes: Eighteen lymph nodes free of metastatic deposits (0/18).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

- Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor.

Verified by :



Patient's Name: Hanan Salah Mohammed Ahmed

Date: 2026-09-03

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Right Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal study.

Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %

0 %

2 %

50 %

98-100%